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Case Number: 23SMCV03853 Hearing Date: July 10, 2026 Dept: O

TENTATIVE RULING

July 10, 2026

23SMCV03853 (Hernandez-Venegas, et. al. vs Coastal
Health Care, Inc., et. al.)

RELEVANT BACKGROUND

This is a wrongful death
action arising from the death of Anna Valarier Casillas Flores (Decedent), a
79-year old woman whose blood sugar became severely and persistently elevated
during her post-operative stay at a nursing facility.

On August 18, 2023, the
complaint was filed by plaintiffs Rita Hernandez-Venegas, Individually and as Successor-In-Interest
to Anna Valarie Casillas Flores (Decedent), Jaime Hernandez, Jr., Anita
Hernandez-Barba, and John Anthony Hernandez (collectively, Plaintiffs)
against defendants Coastal Health Care, Inc. dba Brentwood Health Care Center (Brentwood);
Jonathan Ashcroft (Ashcroft); Robelyn Streegan (Streegan); Manuel Eskildsen (Dr.
Eskildsen); and Does 1-250. Plaintiffs alleged five causes of action: (1) elder
abuse and neglect; (2) negligence; (3) wrongful death (elder abuse and
neglect); (4) wrongful death (negligence); and (5) violation of patient's bill
of rights (Health & Safety Code § 1430 (b)).

On September 25, 2024,
Plaintiffs filed an amendment to the complaint to name Mitchell Taylor (Taylor)
as a Doe defendant.

On May 20, 2025,
Plaintiffs filed an amendment to the complaint to name Adelita T. Cabagnot
(Cabagnot) as a Doe defendant.

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On June 26, 2026,
Plaintiffs' request for dismissal was entered with respect to the first (elder
abuse and neglect), third (wrongful death - elder abuse and neglect), and fifth
(violation of patient's bill of rights (Health & Safety Code § 1430
(b)) causes of action alleged against Eskildsen and Cabaghot.

Settlement Between
Plaintiff and CHC/Taylor

On May 7, 2026, Brentwood
and Taylor (Settling Defendants) filed a Notice of Settlement and an
Application for Determination of Good Faith Settlement. The Court granted the
proposed order regarding the determination of good faith settlement on June 11,
2026. But on June 17, 2026, the Court granted the ex parte application vacating
the June 11, 2026 minute order, deeming the Motion to Challenge timely filed
nunc pro tunc and setting the matter for hearing.

Factual Background

Decedent was admitted to
Brentwood for "short term" rehabilitation on August 10, 2022. (UMF 1.) She had
been diagnosed with: 1) wedge compression fracture of 3rd lumbar; 2)
hypertensive chronic kidney disease; 3) Type 2 Diabetes with diabetic chronic
kidney disease; 4) Type 2 Diabetes with hyperglycemia; and 5) long term
(current) use of insulin, etc. She also presented to Brentwood with a urinary
tract infection, muscle weakness, history of falls, and osteoporosis – among
other conditions. (UMF 2.) Her medication list included blood glucose
monitoring and insulin administration. (UMF 3.)

At the time of
Decedent's admission on August 10, 2022, the Medical Director at Brentwood, Dr.
Eskildsen, was on vacation and did not return until August 15, 2022. (UMF 4, 38.)
From August 10 through 15, 2022, Dr. Susan Leonard (Dr. Leonard) and nurse Cabagnot

were covering for Dr. Eskildsen while he was away on vacation. (UMF 18.)

On August 10, 2022, Decedent was seen by Dr. Leonard for an initial assessment and physical examination. (UMF 5.) Dr. Leonard noted that Decedent had Type 2 diabetes (insulin dependent) with diabetic nephropathy and retinopathy, hypertension, atrial fibrillation and had undergone compression fracture surgery following a fall at home; Decedent underwent an L3 laminectomy with L1-5 percutaneous instrumentation on August 4, 2022. (Id.) Dr. Leonard also noted that, post-operatively, Decedent developed hyperglycemia likely in the setting of steroids or her underlying diabetes. (Id.) Dr. Leonard ordered various medications, including both short-acting and long-acting insulin. (Id.)

On August 12, 2022,
Decedent was seen by Cabagnot. (UMF 19-23.)

On August 15, 2022, Dr. Eskildsen saw and evaluated Decedent. (UMF 24.) Dr. Eskildsen reviewed Decedent's medical records and communicated with Dr. Leonard before seeing Decedent, and he testified that he would have received a brief sign out, including any medical issues involving the patient. (UMF 25-26.) Dr. Eskildsen opted to increase Decedent's insulin dose (long-acting insulin from 22 units to 25 units; short-acting insulin from 12 units to 14 units) to get her blood sugar under better control. (UMF 28.)

On August 18, 2022,
Cabagnot saw Decedent for an unplanned visit when Cabagnot was at Brentwood to see other patients. (UMF 32.) Cabagnot saw a lab result showing possible dehydration and ordered a saline IV and a repeated blood test. (UMF 33.) Decedent's blood glucose was 400. (UMF 28.)

On August 19, 2022,
Decedent had markedly elevated blood sugar. (UMF 48.) Decedent rapidly declined, developing multi-organ failure. (UMF 53.) Decedent was transferred to Providence Saint John's with a blood glucose of 842, a venous blood gas pH of 6.5, septic-shock physiology, and acute encephalopathy. (UMF 28.)

On August 21, 2022,
Decedent passed away. (UMF 53.) Moving Defendants contend that the cause of death was overwhelming sepsis leading to multi-organ failure and that the

elevated blood sugar was a consequence of sepsis rather than the cause of the sepsis. (Id.) Plaintiffs dispute this, stating that the proper framing is not sepsis instead of diabetic ketoacidosis, but a combined process that timely transfer to a hospital would have addressed. (Id.)

INSTANT MOTIONS

On April 15, 2026, Moving Defendants filed the instant motion for summary judgment. Plaintiffs filed an opposition, and Moving Defendants filed a reply. In light of the dismissal entered on June 26, 2026, the Court need only address the remaining second (negligence) and fourth (wrongful death - negligence) causes of action alleged against the Moving Defendants.

On June 1, 2026, Moving Defendants filed their instant motion to challenge the application for determination of good faith settlement between Plaintiffs and Settling Defendants Brentwood and Taylor. Plaintiffs and Settling Defendants filed oppositions, and Moving Defendants filed replies.

DISCUSSION

I.

MOTION

FOR SUMMARY JUDGMENT

“The elements of a cause of action for wrongful death are a tort, such as negligence, and resulting death.” (Lopez v. City of Los Angeles (2011) 196 Cal.App.4th 675, 685.)

In order to state a claim for negligence, a plaintiff must allege the elements of (1) “the existence of a legal duty of care,” (2) “breach of that duty,” and (3) “proximate cause resulting in an injury.” (McIntyre v. Colonies-Pacific, LLC (2014) 228 Cal.App.4th 664, 671.)

“The elements of a cause of action for medical malpractice are: (1) a duty to use such skill, prudence, and diligence as other members of the profession commonly possess and exercise; (2) a breach of the duty; (3) a proximate causal connection between the negligent

conduct and the injury; and (4) resulting loss or damage.” (Chakalis v. Elevator Solutions, Inc. (2012) 205 Cal.App.4th 1557, 1571; See also Lattimore v. Dickey (2015) 239 Cal.App.4th 959, 968.)

The duty of a medical professional is to “possess and exercise, in both diagnosis and treatment, the reasonable degree of knowledge, skill, and care which is ordinarily possessed and exercised by other members of his profession in similar circumstances.” (Landeros v. Flood (1976) 17 Cal.3d 399, 408; Mann v. Cracchiolo (1985) 38 Cal.3d 18, 36.)

Whether a medical professional’s action or inaction was a breach of the professional standard of care can be proven only by expert testimony. (Code Civ. Proc., § 340.5; Sinz v. Owens (1949) 33 Cal.2d 749, 753; see also Louie v. Chinese Hospital Assn. (1967) 249 Cal.App.2d 774, 785.)

As for causation, “[t]he law is well settled that in a personal injury action causation must be proven within a reasonable medical probability based upon competent expert testimony.” (Jones v. Ortho Pharmaceutical Corp. (1985) 163 Cal.App.3d 396, 402.)

For medical malpractice cases, “[w]hen a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence.” [Citations.]” (Hanson v. Grode (1999) 76 Cal.App.4th 601, citing Munro v. Regents of University of California (1989) 215 Cal.App.3d 977, 984–985; See also Borrayo v. Avery (2016) 2 Cal.App.5th 304.)

“In order to testify as an expert in a medical malpractice case, a person must have enough knowledge, learning and skill with the relevant subject to speak with authority, and he or she must be familiar with the standard of care to which the defendant was held. (Evid.Code, § 720, subd. (a); Ammon v. Superior Court (1988) 205 Cal.App.3d 783, 790–791.) An expert may base his or her opinion on any matter reasonably relied upon by experts in forming opinions about the particular subject matter in question, except when the law precludes consideration of a particular matter. (Evid.Code, § 801, subd. (b).)” (Avivi

v. Centro Medico Urgente Medical Center (2008) 159 Cal.App.4th 463, 467.)

Here, Moving Defendants seek summary judgment on the basis that they met the standard of care and that nothing they did caused the Decedent's death, which they attribute to overwhelming sepsis that could not have been prevented. In support, Moving Defendants submit the declarations of their experts, Dr. Mark Sockell (Dr. Sockell) and Sonia Rakkar, MS, BSN (Nurse Rakkar). Moving Defendants' experts opine that they met the standard of medical care and treatment and that no act or omission by Moving Defendants caused Decedent's death.

In opposition, Plaintiffs submit the competing declarations of their experts, Dr. Karen Josephson (Dr. Josephson) and Tammie Hensley, APRN, FNP-C (Nurse Hensley). Plaintiffs' experts opine that Moving Defendants breached the standard of care and that their failures were a substantial factor in causing the decedent's death, that Decedent's condition required immediate hospitalization, and that timely transfer would, more likely than not, have saved her life. Lastly, Plaintiffs state that, where the parties' experts disagree on the standard of care and on medical causation, a triable issue of fact exists, and summary judgment must be denied. In support, Plaintiffs cite *Powell v. Kleinman* (2007) 151 Cal.App.4th 112, 125–126 and *Hanson*, *supra*, 76 Cal.App.4th 601.

In reply, Moving Defendants argue that the expert declarations submitted by Plaintiffs in opposition are speculative. Specifically, Moving Defendants state that Plaintiffs' experts speculate, but have no evidence, that Decedent "developed hyperglycemia" and a systemic infectious process, which ultimately caused her death due to diabetic ketoacidosis. Moving Defendants assert that this contention ignores evidence that Decedent's surgical wound site showed no redness or signs of infection during examinations and discounts that Moving Defendants appropriately addressed Decedent's elevated blood sugar levels with increases in prescribed insulin and adjustments to the sliding scale. Moving Defendants also emphasize that Plaintiff's experts avoid any discussion of notable conditions or presentation which could have alerted the Moving Defendants that Decedent's hyperglycemia was foreseeably fatal and offer no explanations regarding which hospital-level medical interventions they believe could have prevented Decedent's death.

1.

Moving

Defendants' Burden

The

Court finds that Moving Defendants have met their burden as the moving party by submitting the declaration of qualified experts, Dr. Sockell and Nurse Rakkar.

Dr.

Sockell is an Internal Medicine and Geriatrics doctor currently licensed to practice medicine in the State of California and has been in practice for almost 40 years. (Sockell Decl. ¶ 1.) Dr. Sockell is board-certified by the Board of Geriatrics and is the Medical Director and attending physician at the Novato Healthcare Center, a 180-bed Skilled Nursing Facility located in Novato, California. (Id. ¶ 5.) Based upon the foregoing background, training, and experience, he is familiar with the standard of care required of the Moving Defendants in practicing geriatrics and internal medicine in California and has personal knowledge of the matters set forth herein. (Id. ¶¶ 3-6.)

Based

upon Dr. Sockell's review of the evidence in this case, including the operative pleadings, the relevant medical records, discovery responses, and deposition testimony, as well as his education, training and experience, it is his opinion, to a reasonable degree of medical probability, that all of the care and treatment rendered to Decedent by the Moving Defendants was appropriate and within the standard of care within the community. (Id. ¶¶ 7-11.) Dr. Sockell opines that Dr. Eskildsen did not breach any duties regarding the clinical policies as Medical Director of Brentwood. (UMF 38, 41, and 44.) Furthermore, Dr. Sockell declares that the care and treatment rendered to Decedent by the Moving Defendants was appropriate and within the standard of care within the community, including with regard to medication management and nursing orders. (UMF 45.)

Dr.

Sockell declares the following, in pertinent part:

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Decedent had brittle diabetes with wide fluctuations in her blood sugar that was evident when she experienced post-operative hyperglycemia that was difficult to manage, even prior to her

admission to Brentwood's nursing facility, the Moving Defendants appropriately responded to Brentwood's nursing reports of continued elevated blood sugars. (UMF No. 46; Sockell Decl., at ¶11(b).) As such, Dr. Sockell concludes the Moving Defendants made appropriate increases to the insulin regimen. (UMF 46; Sockell Decl., at ¶11(b).)

.
Cabagnot's examination, assessment and treatment of Decedent on August 12, 2022, met the standard of care because, to better control the blood sugar, it was reasonable for Cabagnot to increase the dosage of Decedent's long-acting and short-acting insulins and to order repeat blood work for August 15, 2022. (UMF No. 49; Sockell Decl., at ¶11(c).)

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Dr. Eskildsen's evaluation, assessment and treatment of Decedent on August 15, 2022, also met the standard of care since Decedent's blood sugars remained elevated despite the previously ordered increase in insulin dosage; it was reasonable and not below the standard of care for Dr. Eskildsen to further increase the dosage of Decedent's long-acting and short-acting insulins. (UMF No. 50; Sockell Decl., at ¶11(d).)

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Cabagnot's examination, assessment, and treatment of Decedent on August 18, 2022, met the standard of care. It was reasonable and not below the standard of care for Cabagnot to order saline IV to address Decedent's suspected minor dehydration, to order Zofran to address Decedent's reported nausea, and to order further blood tests on the following day. (UMF No. 51; Sockell Decl., at ¶11(e).)

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The Decedent was given appropriate treatment and follow-up care; Dr. Eskildsen's instruction to notify the attending and/or on-call physician regarding any blood sugar readings outside of range was within the standard of care; and Cabagnot's orders regarding blood tests were within the standard of care. (UMF No. 52; Sockell Decl., at ¶11(f).)

(Sockell Decl. ¶ 11.)

On
the issue of causation, Dr. Sockell declares that nothing in the medical

records indicates that any conduct or omission by Moving Defendants caused or contributed to Decedent's death. Dr. Sockell explains that Decedent's markedly elevated blood sugar on August 19, 2022, was due to overwhelming sepsis and could not have been prevented. (UMF No. 48; Sockell Decl., at ¶11(b).) Dr. Sockell further explains that the elevated blood sugar of 715 mg/dl on August 19, 2022, was a consequence of – and not the cause of – overwhelming sepsis. (Id.) Lastly, Dr. Sockell explains that Decedent's medical records from St. John's Hospital indicate that she had extreme lactoacidosis, which is a sign and symptom of sepsis and acute kidney injury – not diabetic ketoacidosis. (UMF No. 57; Sockell Decl., at ¶12(c).)

Nurse

Rakkar is a Registered Nurse with a Master's in Nursing, currently a licensed nurse in the State of California, and has over 15 years of experience in both skilled nursing facility and home health. (Rakkar Decl., at ¶¶ 1, 3-4.) Nurse Rakkar is supportive of Dr. Sockell's opinions, and opines that, based upon her own review of the evidence in this case, including the operative pleadings, the relevant medical records, discovery responses and deposition testimony, as well as her education, training and experience, all of the care and treatment rendered to Decedent by the Moving Defendants was appropriate and within the standard of care within the community with regard to medication management and nursing orders. (Rakkar Decl., at ¶¶ 5-8, 10.)

2. Plaintiffs' Burden

The

Court finds that Plaintiffs have met their responsive burden as the non-moving party to raise a triable issue of material fact by submitting the declarations of their experts, Dr. Josephson and Nurse Hensley.

Dr.

Josephson has been board certified by the American Board of Internal Medicine since 1991, with subspecialty certification in Geriatric Medicine since 1994. (Josephson Decl. ¶ 2.) Dr. Josephson has served as an attending physician for patients admitted to skilled nursing facilities for post-acute rehabilitation, including patients with insulin-dependent diabetes and post-operative recovery needs comparable to those of the decedent in this action. (Id. ¶ 3.)

Based

upon Dr. Josephson's review of the evidence in this case, including the

operative pleadings, the relevant medical records, discovery responses and deposition testimony, he opines that Moving Defendants breached the standard of care and that their failures were a substantial factor in causing the decedent's death; that Decedent's condition required immediate hospitalization; and that timely transfer would, more likely than not, have saved her life.

With respect to the standard of care, Dr. Josephson declares the following, in pertinent part:

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The standard of care required Dr. Eskildsen, having on August 15 increased the patient's insulin in response to severely elevated glucose values, to (i) establish a defined plan for re-evaluation of the patient's glycemic response to the increased insulin within 48 to 72 hours, (ii) leave clear instructions so that sustained glucose values above the sliding-scale threshold would be promptly communicated to him or to a covering physician with prescriptive authority, and (iii) personally re-evaluate the patient, or arrange for her re-evaluation by another physician in his coverage group, in light of the persistent severe hyperglycemia. Dr. Eskildsen did none of these things. His failure to do so was a breach of the standard of care. (Id. ¶ 22.)

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Decedent's chart suggests that her attending physician was contacted regarding her elevated blood sugar values. The Medication Administration Records reflect that, upon elevated blood sugar results, the decedent received a rapid-acting dose of Aspart or, at the least, that the nursing staff recorded their name and the blood sugar result. In addition, Dr. Eskildsen's increase of the decedent's baseline insulin doses at his August 15, 2022, evaluation demonstrates that he had reviewed her prior blood sugar results from August 10 through August 15, 2022. (Id. ¶ 25.)

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The same failure of oversight is reflected in the revised sliding-scale order that appeared in the chart on August 18, 2022, raising the physician-notification threshold from 330 to 401 mg/dL, on the same day the decedent's glucose reached 571 and within hours of her decompensation. Raising a notification threshold in

a patient whose glucose was escalating is the opposite of what the standard of care required; it made it less likely, not more, that a dangerous value would reach a physician. (Id. ¶ 27.)

Cabagnot's

afternoon evaluation of the decedent on August 18, 2022. represents the clearest and most consequential breach of the standard of care in this case. At that time, the following conditions were present: (a) A blood glucose of 571 mg/dL that morning, the highest reading of the admission, far above the sliding-scale notification threshold; (b) A reading of 534 mg/dL the prior morning; (c) A pattern of glucose values that were rising rather than falling, despite Dr. Eskildsen's upward titration of insulin on August 15; (d) August 15 laboratory results showing possible dehydration and hyponatremia (serum sodium 127–128 mEq/L), which was the reason the nurse brought the labs to her attention, and which, in an insulin-dependent diabetic, was itself a marker of severe hyperglycemia that should have prompted immediate inquiry into the patient's blood glucose; (e) Nausea sufficient to warrant a Zofran order; (f) A recent surgical incision that the orthopedic surgeon had identified two days earlier as having "incisional redness," for which Keflex had been initiated; and (g) An elderly insulin-dependent diabetic with multiple comorbidities and post-operative status whose clinical picture had not improved over the preceding several days. Dr. Josephson states that these findings, in an elderly insulin-dependent post-operative diabetic patient, are a clinical alarm consistent with developing diabetic ketoacidosis and an unresolved or developing infection driving worsening hyperglycemia. (Id. ¶ 28-29.)

The

standard of care required Cabagnot to: (i) Before issuing orders, review the patient's available chart and recent blood glucose values, inquire of the nursing staff as to the patient's recent glycemic trend, and obtain an immediate point-of-care fingerstick blood glucose, particularly given the nausea and possible dehydration that were the stated reason for the encounter; (ii) Recognize the constellation of findings as a clinical alarm requiring hospital-level care, regardless of whether the underlying process was DKA, infection, both, or another serious process; (iii) Personally notify the attending physician, Dr. Eskildsen, of the clinical picture and her impression; (iv) Arrange for the decedent's prompt transfer to an acute care hospital capable of providing intravenous insulin therapy, aggressive intravenous fluid

resuscitation, empiric intravenous antibiotic therapy, intensive monitoring, and the laboratory work-up necessary to clarify the underlying process; (v) Initiate intravenous fluid resuscitation as a temporizing measure pending transfer; and (vi) Document her bedside evaluation, findings, clinical impression, differential diagnosis, and plan contemporaneously in the chart. Cabagnot did none of these things. (Id. ¶ 29.)

(Josephson
Decl. ¶ 22-27.)

With respect to causation, Dr.
Josephson declares the following, in pertinent part:

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To
a reasonable degree of medical probability, the decedent's death resulted from the failure of the Moving Defendants, the UCLA Health clinicians responsible for her medical care, to appreciate, diagnose, and treat her impending diabetic ketoacidosis and the systemic process driving it, and that failure was a substantial factor in causing her death regardless of how the contributions of the diabetic ketoacidosis and the infection are ultimately apportioned. (Id. ¶ 34.)

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To
a reasonable degree of medical probability, had the decedent been transferred to a hospital on the afternoon of August 18, 2022, before her cascade of organ failure had advanced to the point of irreversibility, her DKA would have been treated with intravenous insulin and fluids, any infection would have been treated with empiric intravenous antibiotics, and she would more likely than not have survived. The Providence record confirms that her DKA responded to treatment when treatment was provided; what ultimately killed her was the delay in providing it. (Id. ¶ 40.)

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To
a reasonable degree of medical probability, had Dr. Eskildsen, after his August 15 evaluation, established a follow-up plan and ensured that the rising glucose values reached a physician who could act on them, the decedent's deteriorating clinical picture would have been recognized and addressed in time to prevent

her death. (Id. ¶ 41.)

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To

a reasonable degree of medical probability, had NP Cabagnot, on the afternoon of August 18, recognized the clinical picture in front of her, notified the attending physician, and arranged for the decedent's prompt transfer, the decedent would more likely than not have survived. (Id. ¶ 41.)

(Josephson

Decl. ¶ 34-41.)

Additionally, with respect to the

Decedent's overall condition and cause of death, Dr. Josephson declares the following:

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Decedent's

blood sugars became severely and persistently elevated during her post-operative course beginning at UCLA and continuing at Brentwood, a derangement that her UCLA records attribute in part to perioperative steroid (dexamethasone) administration and that, as discussed below, was further driven by a developing systemic process superimposed on her diabetes. The severe and persistent hyperglycemia she experienced in August 2022 was therefore not the stable baseline state of her diabetes but an acute and worsening derangement that called for recognition and treatment.

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Upon

arrival at Providence Saint John's on August 19, 2022, the decedent was triaged hypothermic, tachypneic, hypoxic, and with acute encephalopathy. She was so dehydrated that initial intravenous access could not be obtained and an intraosseous line was required. (Yomtoubian ED records, P-17 through P-26.)

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The

treating physicians at Providence identified diabetic ketoacidosis as a principal admitting diagnosis. The ICU intensivist identified the principal admitting problem as diabetic ketoacidosis, the emergency physician documented that the decedent presented in DKA with a blood glucose of 842, a bicarbonate

of 4, and an anion gap of 30; and the infectious disease consultant likewise listed DKA with severe metabolic acidosis in her impression. (Id. ¶ 14; Providence records, P-60, P-82, P-23, P-87.) The Certificate of Death, signed by Dr. Puscas, identifies the immediate cause of death as “DIABETIC KETOACIDOSIS,” “DIABETES MELLITUS TYPE I,” with disseminated intravascular coagulation, acute liver failure, and acute renal failure listed as other significant conditions contributing to death. (P-5.)

(Josephson
Decl. ¶ 8-13.)

Additionally, Nurse Hensley is a board-certified Family Nurse Practitioner and Advanced Practice Registered Nurse, duly licensed to practice in the State of California who has been practicing for thirteen years as an emergency department Nurse Practitioner, currently as Lead Emergency Department Nurse Practitioner. (Hensley Decl. ¶ 1-3.)

Nurse Hensley opines that, based on Decedent's presentation on the afternoon of August 18, 2022, together with her underlying conditions, it is her opinion to a reasonable degree of probability that her condition required immediate hospitalization on an emergency basis. (Id. ¶ 11.) Nurse Hensley further opines that, had the decedent presented to an emergency department, or been transferred to one, on August 18, 2022 with the condition described above, she would have been hospitalized. (Id. ¶ 13.) Had the Decedent instead been transferred to an emergency department at the time of her afternoon evaluation on August 18 – roughly twelve to fourteen hours before she actually reached the hospital – she would have received intravenous insulin, aggressive intravenous fluids, and empiric intravenous antibiotics before her cascade of multi-organ failure became irreversible, and she more likely than not would have survived. (Id. ¶ 14.) Cabagnot, who evaluated the decedent at the bedside on the afternoon of August 18, 2022, and was about to issue medical orders, should have known the blood glucose levels documented in the chart, which were over 500 mg/dL. Those values established the need to send this patient to the hospital immediately, and the failure to do so was a breach of the standard of care for NP Cabagnot. (Id. ¶ 15.)

The
Court disagrees with Moving Defendants' contention that Dr. Josephson's and Nurse Hensley's competing expert opinions are speculative. As set forth above, both expert opinions are well-reasoned and provide a thorough explanation in

support. (See Josephson Decl., generally; Hensley Decl., generally.) The Court finds that the competing declaration of Plaintiffs' experts raise triable issues of material fact as to whether Moving Defendants breached the standard of care and caused Decedent's death.

Accordingly,

Moving Defendants' motion for summary judgment or adjudication is DENIED.

II.

CHALLENGE

TO DETERMINATION OF GOOD FAITH SETTLEMENT

Code of Civil Procedure section

877.6, subdivision (b) provides that, upon receiving an application for a determination of good faith settlement between the settling party and the plaintiff, a non-settling alleged joint tortfeasor defendant may file a motion to contest the good faith of the settlement. (Code Civ. Proc., § 877.6, subd. (b).)

"The ultimate determinant of good

faith is whether the settlement is grossly disproportionate to what a reasonable person at the time of settlement would estimate the settlor's liability to be." (City of Grand Terrace v. Superior Court (1987) 192

Cal.App.3d 1251, 1262.) "Equity is the aim of section 877.6...equitable sharing of costs among the parties at fault and encouragement of settlements." (Long Beach Memorial Med. Center v. Superior Court (2009) 172 Cal.App.4th 865, 872.) A "settlement figure must not be grossly disproportionate to what a reasonable person, at the time of the settlement, would estimate the settling defendant's liability to be." (Tech-Bilt, Inc. v. Woodward-Clyde & Associates (1985) 38 Cal.3d 488, 499.)

In Tech-Bilt, the

California Supreme Court identified the following nonexclusive factors courts are to consider in determining if a settlement is in good faith under section 877.6: "a rough approximation of plaintiffs' total recovery and the settlor's proportionate liability, the amount paid in settlement, the allocation of settlement proceeds among plaintiffs, and a recognition that a settlor should pay less in settlement than he would if he were found liable after a trial. Other relevant considerations include the financial conditions and insurance policy limits of settling defendants, as well as the existence of

collusion, fraud, or tortious conduct aimed to injure the interests of nonsettling defendants.” (Tech-Bilt, supra, 38 Cal.3d at p. 499.)

“[A] court not only looks at the alleged tortfeasor's potential liability to the plaintiff, but it must also consider the culpability of the tortfeasor vis-à-vis other parties alleged to be responsible for the same injury. Potential liability for indemnity to a nonsettling defendant is an important consideration for the trial court in determining whether to approve a settlement by an alleged tortfeasor.” (TSI Seismic Tenant Space, Inc. v. Superior Court (2007) 149 Cal.App.4th 159, 166, citing West v. Superior Court (1994) 27 Cal.App.4th 1625, 1636–1637 [reversed the determination of good faith settlement, reasoning that the trial court should have considered the culpability of the settling brokers and their potential liability for indemnity to the non-settling sellers].)

Here, the settlement terms at issue are as follows: “Without admitting any fault or liability, Defendants, Coastal Health Care, Inc. dba Healthcare Center and Mitchell Taylor, have agreed to resolve this matter with Plaintiffs for payment of \$525,000.00 (Five Hundred Twenty-Five Thousand Dollars) with each party to bear their own costs and attorney's fees, in exchange for entry of dismissal with prejudice of the action as to the above-mentioned Defendants.” (Application for GFS, at p. 5.)

Moving

Defendants argue that the \$525,000 settlement does not fall within the reasonable range of the settling parties' proportionate liability based on the Tech-Bilt factors. In support, Moving Defendants identify the categories of damages placed at issue by the pleadings and governing law, including potential exposure for elder abuse, punitive damages, and statutory attorney's fees.

Specifically,

Moving Defendants contend that (1) Brentwood, not Moving Defendants, bore custodial responsibility for Decedent's daily care, including administration of medication, monitoring, and activities of daily living as defined under Welfare and Institutions Code section 15610.57; (2) the settlement fails to reasonably account for the significant disparity between Settling Defendants' alleged custodial exposure and Moving Defendants' limited, episodic clinical involvement; (3) the record demonstrates a materially different scope of duty and comparative exposure between Brentwood and Moving Defendants that is not

reflected in the settlement allocation; and (4) Plaintiffs' mediation demands to Moving Defendants, while contextually relevant, further underscore the disproportion between the negotiated settlement amount and the alleged allocation of responsibility among defendants when viewed within the Tech-Bilt "ballpark" framework.

Moving

Defendants emphasize that Plaintiffs' operative allegations concern Brentwood's custodial failures, including repeated failures to administer insulin, maintain hygiene, implement care plans, and provide basic activities of daily living, all of which are statutorily defined forms of "neglect" under Welfare and Institutions Code section 15610.57. Plaintiffs further allege repeated deterioration events, including hyperglycemia, dehydration, incontinence-related neglect, and failure to respond to escalating medical conditions, all occurring while Decedent was under Brentwood's exclusive custodial care. Moving Defendants further contend that the evidentiary record confirms that Settling Defendants —not Moving Defendants—had custody and control of Decedent's daily care, including monitoring blood sugar, administering insulin, and implementing care plans through its nursing staff and supervisory personnel.

In

opposition, Plaintiffs argue that, once a good faith settlement is contested, it is the challenging party – not the settling parties – who bears the burden of proof. (Code Civ. Proc., § 877.6, subd. (d).) To carry that burden, the Moving Defendants must come forward with admissible evidence showing that the \$525,000 settlement is so far "out of the ballpark" as to be inconsistent with the objectives of section 877.6. (Tech-Bilt, *supra*, 38 Cal.3d at pp. 499–500.)

Additionally,

Plaintiffs state that Moving Defendants' motion is internally contradictory because they have moved for summary judgment asserting that they bear no liability whatsoever, and their own retained experts opine that they had no custodial relationship with and no custodial responsibility for the decedent, yet they now claim to be prejudiced because Brentwood – the entity that did have that custodial relationship – supposedly settled for too little. Plaintiffs state that a party that concededly shares none of the settling defendant's liability cannot be heard to complain that the settling defendant paid too little for that liability. Plaintiffs also assert that the settlement causes

the Moving Defendants no cognizable prejudice, and the motion identifies none.

In

opposition, Settling Defendants argue that the settlement was agreed to by the Settling Defendants to avoid the uncertain result of a trial of this matter and to avoid additional litigation fees and costs including those associated with trial. Settling Defendants further state that the settlement is "within the ballpark" pursuant to Code of Civil Procedure §877 and TechBilt.

Settling Defendants also contend that Plaintiffs have the burden of proving the lack of good faith, citing *Fisher v Superior Court* (1980) 103 Cal.App.3d 434, 449.

Also, Settling

Defendants emphasize that Moving Defendants have presented no evidence that the settlement amount agreed to between Plaintiffs and Brentwood/Taylor was not a rough approximation of the Settling Defendants' proportionate liability.

In

reply, Moving Defendants argue that the pleadings, discovery, deposition testimony, and facility records demonstrate that Settling Defendants—not Moving Defendants—were responsible for the custodial care functions at the core of Plaintiffs' Elder Abuse claims, including administration of insulin, hygiene, monitoring, and implementation of care plans.

The

Court finds that the \$525,000 settlement falls within the reasonable range of proportional liability under Code of Civil Procedure section 877.6 and Tech-Bilt, supra, 38 Cal.3d 488. Moving Defendants have failed to demonstrate that the settlement is grossly disproportionate to what a reasonable person at the time of settlement would estimate the Settling Defendants' liability to be. Moving Defendants repeatedly assert that the settlement does not fall within the reasonable range of the Settling Defendants' proportionate liability because Settling Defendants, not Moving Defendants, are responsible for the injuries related to the elder abuse claims. But, as previously stated, Plaintiffs have dismissed the elder abuse claims against Moving Defendants, and therefore there is no proportional liability at issue regarding the elder abuse claims. Additionally, the Court agrees with Plaintiffs and Settling Defendants that the settlement is within the Tech-Bilt ballpark and reasonable in light of the circumstances. Plaintiffs correctly point out that Tech-Bilt expressly

recognizes that a settling defendant should pay less in settlement than it might be ordered to pay after trial. (Tech-Bilt, supra, 38 Cal.3d at p. 499.) Lastly, Moving Defendants have not shown or alleged the existence of collusion, fraud, or tortious conduct aimed to injure the interests of Moving Defendants.

Accordingly, Moving Defendants' motion to challenge the determination of good faith settlement is DENIED. The application for determination of good faith settlement is GRANTED.